

QMS SOP TEMPLATE:

# Product Recall Management

Quality  
Management  
System  
Standard  
Operating  
Procedure





# QMS SOP TEMPLATE: Product Recall Management

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# Introduction

After a product has been placed on the market, any retrieval of it from the distribution network as a result of a quality defect should be regarded and managed as a recall.

Recalls can be initiated on the basis of a complaint, a stability study result, a clinical study result, a reported adverse event or any other investigation that may give an indication that the quality, safety or efficacy of a product on the market might be at risk.

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## 1.0 Purpose

This procedure is intended to describe the methodology for identifying, initiating, and managing a product recall in a prompt and complete manner, and in accordance with applicable regulatory requirements of distributed product that is suspected or known by management to be seriously defective and/or pose potential health risk to end-users.

## 2.0 Scope

This procedure applies to all recalls or possible recalls initiated for a pharmaceutical product distributed (including products used for market studies and clinical trials) and sold to customers and may pose a risk due to a defect, contamination, mislabeling, or other safety or quality concerns.

## 3.0 Definitions/Abbreviations

**Adverse Drug Event (ADE):** Any unfavorable and unintended symptom, occurrence, or experience affecting a clinical trial subject or patient using a marketed drug product.

**Field Alert Report (FAR):** The purpose of the FAR is to quickly identify quality defects in distributed drug products which may present a potential safety threat.

**Recall:** Action taken to remove a drug product or device for human use from the market due to a potential or confirmed safety risk.

**Stock Recovery:** The removal or correction of a product that has not been marketed or has not left the direct control of the manufacturing organization.

**Voluntary Recall:** Action initiated by the company, not mandated by regulatory authorities.

**Class I Recall:** Reasonable probability that use/exposure to the product will cause serious adverse health consequences or death.

**Class II Recall:** Use of, or exposure to, the product may cause temporary or medically reversible adverse health consequences or where the possibility of serious adverse health effects is remote.

**Class III Recall:** Use of, or exposure to the product is unlikely to cause any adverse health consequences.

**Quality Assurance/Qualified Person:** Final responsibility for quality assessment and oversight of the recall process.

## 4.0 Responsibilities

**Quality Head (or Designee):** responsible for ensuring compliance with this procedure. Contacts the responsible authority and advises about the detail of the Recall event, Recall classification and strategy, method of notification and implementation of the recall or field action.

**Recall Coordinator:** coordinates the administrative recall process and maintains the documentation.

**Recall Committee:** Evaluates the need to remove products from the marketplace. Classifies removals as Class I, Class II or Class III, market withdrawal or stock recovery based upon the severity and type of defect, extent of incident, degree of health hazard, and existence of a regulatory violation.

**Logistics Manager:** coordinates the physical recall, receipt, storage and final destruction of product, if required.

**Qualified Person/Quality Assurance:** helps assess risk level, responsible for the quarantine of potentially affected batches and subsequent investigations. Chairs the Recall Committee. Investigates the root cause and recommends corrective/preventive actions. Ensures appropriate final disposition of the affected product to address issues of both safety and regulatory compliance and ensures the proper close out and documentation of the Recall.

## 5.0 Requirements

The requirements in a recall realization shall include, but are not limited to:

- 5.1     *Recall Committee to Evaluate Potential Recalls***
- 5.2     *Classification of Recalls by Severity***
- 5.3     *Establishment of Timeframes Allowed for Executing Different Classes of Product Recall***
- 5.4     *Obtaining Distribution Information for Affected Batches***
- 5.5     *Generating Recall Communication Content and Format***
- 5.6     *Recall Efficacy Evaluations***
- 5.7     *Recall Closeout Documentation***
- 5.8     *Mock Recalls and Evaluation of the Process***
- 5.9     *Provisions for Corrective Actions***

## 6.0 Summary/Flowchart

- 6.1     *Standard Preparation for Recall Before Distribution Takes Place***
- 6.2     *Initiation of a Possible Recall***
- 6.3     *Definition Recall Classification and Strategy***
- 6.4     *Specifics for Recall in the USA or Europe***
- 6.5     *Data that Need to be Available***
- 6.6     *Deciding on Extent of the Recall***
- 6.7     *Communicating the Recall***
- 6.8     *Receiving Recalled Product***
- 6.9     *Closing the Recall***



## 7.0 Procedure

### 7.1 *Standard Preparation Before Distribution Takes Place*

Reason for recall, reasons for considering a recall can be as follows:

- Any complaint, adverse event, investigation, aberrant result in clinical or stability studies can lead to a recall
- An order or request from the Competent Authorities

Before distribution of any pharmaceutical product to clinical sites or to the market, a recall process should be defined, documented, tried and evaluated. Any recommendations following this trial recall should be incorporated into the process.

Required preparation for any recall:

- A Recall Committee including at least the Recall Coordinator, Quality Assurance/Qualified Person, a representative from Regulatory Affairs and, when and where required, Manufacturing, Quality, Clinical Operations, Medical Affairs, Legal Affairs
- A list of emergency contacts that needs to be maintained and available, including all members of the Recall Committee, the Chief Executive Officer, the Competent Authorities
- A Recall Form and Recall log for internal use
- A standard document to inform the Competent Authorities in case of a recall
- A standard document to inform the wholesalers, distributors and patients on a (possible) recall and the consequences
- Batch documentation
- Distribution information
- Possible storage location(s) for recalled product

### 7.2 *Initiation of a Possible Recall*

In case of a possible recall:

- The Recall Coordinator initiates the first part of the Recall Form and notifies the Qualified Person/Quality Head
- Qualified Person/Quality Head may decide, to start a recall before any further investigation into the root cause is performed
- Otherwise, the Recall Coordinator arranges a meeting with the Recall Committee on the shortest possible notice
- The Recall Committee will review the event and will make a recommendation to recall or not, with the Quality Assurance/QP making the decision
- If the decision is made to recall, all stock available will be blocked and transferred to a designated and closed location

### 7.3 Definition Recall Classification and Strategy

The Recall Committee shall evaluate the need to remove products from the market, classify a removal as a Class I, Class II or Class III market withdrawal or stock recovery based upon the severity and type of defect present or the extent of the reported incident, the degree of the health hazard, when applicable, and the existence and severity of the violation. A Field Alert Report (FAR) is required to be submitted to the FDA within three (3) business days for any significant problem with an approved and distributed drug product

The Recall Committee shall develop a recall strategy, which shall address the extent of the product recall.

A recall strategy consists of:

1. Description of Product Identity
  - Brand name, Size(s) and Type(s) of containers
  - Dosage strength(s) and pharmaceutical form(s)
  - Contact details with contact person of the firm on the label
  - Lot number(s)
  - Date(s) of production and production location(s)
2. Estimated amount on the market
  - Amount manufactured
  - Amount distributed, date(s) of distribution
  - Amount in possession at manufacturer and manufacturer's distribution facilities
  - Level of distribution and type of recipient (wholesaler, pharmacy, user, etc.)
  - All area(s) and countries of distribution
  - Estimated total amount on the market
3. Estimated range of usage
4. Reason(s) for recall, date(s) of recall and circumstances of recall
5. The Recall Committee shall classify the recalls based on the extent of the potential health hazards and risks to patients as identified by Medical Affairs. The following shall be used when assigning the appropriate Case Classification
6. Key Factors:
  - Health Hazard:
    - Injuries or disease already caused:
    - Hazards to varying segments of the population

- Seriousness of health hazards to exposed persons
- Immediate and long-term consequences
- Extent of Recall:
  - Results of Health Hazard Evaluation:
  - Ability to trace the product in the market
  - Degree of the product's deficiency or violation
  - Degree to which the product remains in the market
  - Continued availability of the product for essential medical needs
- Recall Committee Meeting:
  - The following at a minimum, shall be assessed by the Recall Committee:
    - > Extent of the recall: Is the potential recall limited to one country, or does it have potential impact on other countries/regions?
    - > Technical background information on which the assessment is based, and which can be immediately shared with other countries/regions, if necessary.
    - > Coordination of Corrective Action plans and determination if a regulatory notification (i.e., FAR) is required, and if not already completed.
    - > Determination of which warnings, if any, to recommend to the regulatory authorities, local or national news media, specialized trade press, or warnings to specific segments of the population (e.g., physicians, clinics, and/or hospitals).

#### **7.4 Recall in the USA or Europe**

In case a product is marketed in the United States of America, and a recall is required, please refer for recommendations to the FDA "Guidance for Industry, Product Recalls, Including Removals and Corrections." Use for notification of the Authorities: "Fast Track", through the internet, [www.saferproducts.gov/business](http://www.saferproducts.gov/business).

If the product is marketed in the EU, please refer to the EMA website for further instructions Quality Defects and Recalls | European Medicines Agency (EMA), [www.ema.europa.eu/en/human-regulatory-overview/post-authorisation/compliance-post-authorisation/quality-defects-recalls](http://www.ema.europa.eu/en/human-regulatory-overview/post-authorisation/compliance-post-authorisation/quality-defects-recalls).

When the recall is reported to one member state of the EU, the Competent Authorities will notify all other member states.

#### **7.5 Data Readily Available in Order to Perform a Recall**

To the personnel responsible for recalls, the following should be readily available:

- The batch/product distribution records with sufficient information on wholesalers and directly supplied customers (with addresses, phone and/or fax numbers inside and outside working hours, batches and amounts delivered), including those for exported products and medical samples.

- For investigational medicinal products, all trial sites and the countries of destination. When a marketing authorization has been issued, the manufacturer, in cooperation with the sponsor, should inform the marketing authorization holder of any quality defect that could be related to the product. The sponsor should be able to perform rapid unblinding of blinded products, in case this is necessary for a prompt recall and should ensure that the procedure discloses the identity of the blinded product only as far as necessary.

## **7.6      *Extend of the Recall in Discussion with the Competent Authorities***

After consultation with the concerned Competent Authorities, it should be decided as to how far into the distribution network a recall should extend, taking into account:

- The potential risk to public or animal health and any impact that the proposed recall action may have.
- The therapeutic use of the product in combination with the risk of shortage of a medicinal product which has no authorized alternative.

Any decisions not to execute a risk-reducing action which would otherwise be required should be agreed with the Competent Authorities in advance.

The Competent Authorities should also be informed in situations in which no recall action will take place because the batch has expired (in products with short shelf-life).

If a proposed recall may affect different markets in different ways, appropriate market-specific risk-reducing actions should be developed and discussed with the concerned competent authorities.

All concerned Competent Authorities should be informed in advance in cases where products are intended to be recalled. For very serious issues (i.e., those with the potential to seriously impact upon patient or animal health), rapid risk-reducing actions (such as a product recall) may have to be taken as soon as possible. However, attempts should always be made to agree in advance with the Competent Authorities.

## **7.7      *Communicating the Recall to Distributors, Wholesalers, and Patients***

Once a recall strategy has been decided and agreed with the Competent Authorities, official communication to return the product will be sent out to those who have received it, including instructions on how to return and what the compensation will be.

## **7.8      *Receiving Recalled Product***

Upon receipt of the recalled product, the product will be inspected, counted and securely stored in a designated and closed area. If required through the recall strategy or any other investigation, samples may be taken for further investigation.

The number of received products will be tracked on a regular basis to monitor progress and this will be reported to the Competent Authorities, on a frequency as previously agreed. Also, any difficulties in retrieving the product or new information regarding the product defects will be shared with the Competent Authorities.

## **7.9      *Closing the Recall***

When as much product as still was available is withdrawn from the market, the recall reconciliation can take place. All discrepancies from a full reconciliation need to be investigated and explained.

The results of the investigation into the quality problem and associated CAPA's must be documented in a recall report that will be shared with the Competent Authorities.

If there are no additional requirements that may arise from discussing the report with the Competent Authorities, the recall may be closed by the Quality Assurance/Quality Person as well as the decision on the recalled material is made.

## 8.0 Calculations

Calculations to monitor the actual return of recalled product are required to monitor the progress of the recall initiated. See also the Recall Form.

## 9.0 Documentation of Results

- All activities related to recalls are documented on a Recall Form, per Recall, with a standard Recall Form as a basis.
- A medical assessment prepared describing the potential patient risk based on a scientific assessment.
- A technical assessment containing, at a minimum, a detailed description of:
  - The non-compliance, product deficiency or violation
  - How the non-compliance was detected
  - The root cause of the non-compliance, as clearly and with as much depth to allow a complete understanding of the situation
  - Any corrective actions, immediately taken to prevent recurrence of the non-compliance, product deficiency or violation
  - An evaluation of other batches or other products which may have been affected by the recall
  - A list of affected batches
- All summary reports, forms, records, etc., documenting the analysis and the execution of a recall shall be maintained by the Quality Unit in perpetuity

## 10.0 Recall Closeout

- Each recall shall be officially closed when there is adequate documented proof that all affected product has been recovered.
- The recall may be closed out only when the following has been satisfied:
  - The returned product quantities are in line with the original expected volume of recalled product.
  - The destruction of the recalled product is complete and documented.
  - If required, the official closeout shall be communicated to the appropriate regulatory authorities.

## 11.0 Mock Recall

- The product recall procedure for the commercial products shall be tested no less than once every two (2) years if no real recall occurred during that time.
- Testing of the product recall procedure shall, at a minimum, include:
  - The retrieval of information concerning distributed batches
  - Disposability, cooperation, and decision-making process of the Recall Committee.
  - The creation of mock communication package for regulatory agencies including the letters informing agencies and clients about the recall
  - The creation of mock internal documentation and communication package

## 12.0 References

- 21 CFR Part 210 – Current Good Manufacturing Practice in Manufacturing, Processing, Packing, or Holding of Drugs; General
- 21 CFR Part 211 – Current Good Manufacturing Practice for Finished Pharmaceuticals
- EU GMP Guide – Part I: Chapter 1 (Pharmaceutical Quality System)
- Pharmaceutical Inspection Co-operation Scheme. *PIC/S Guide to Good Manufacturing Practice for Medicinal Products: Part I (PE 009-17)*
- PIC/S Procedure for Handling Rapid Alerts and Recalls Arising from Quality Defects (PI 010-5)
- TRS 986 – Annex 2: WHO Good Manufacturing Practices for Pharmaceutical Products: Main Principles

For a Rapid Alert Notification Form (EU) and Follow-up and non-urgent information for Quality Defects Form (EU), see Quality Defects and Recalls | European Medicines Agency (EMA), [www.ema.europa.eu/en/human-regulatory-overview/post-authorisation/compliance-post-authorisation/quality-defects-recalls#related-eu-legislation-18752](http://www.ema.europa.eu/en/human-regulatory-overview/post-authorisation/compliance-post-authorisation/quality-defects-recalls#related-eu-legislation-18752).

## 13.0 Revision History

- Rev 01 – Initial release
- Rev 02 – [To be updated with future changes]

## 14.0 Addenda

### 14.1 *Addendum I: Classification of Recalls Rapid Alert System – Urgency of Defective Medicinal Product Alerts (from PI 010-05)*

#### Class I

Class I defects are potentially life threatening or could cause a serious risk to health. These must be notified through the Rapid Alert System in all cases.

Examples:

- Incorrect product (label and contents are different products)
- Correct product but wrong strength, with serious medical consequences
- Microbial contamination of sterile injectable or ophthalmic product
- Chemical contamination with serious medical consequences
- Mix-up of some products (rogues) with more than one container involved
- Incorrect active ingredient in a multi-component product, with serious medical consequences.

#### Class II

Class II defects could cause illness or mistreatment, but are not Class I. A rapid alert notification should be sent to all contacts of the rapid alert notification list as it might be difficult to know where a batch has been distributed. If the product distribution is known, the notification should be only sent to the contacts concerned.

Examples:

- Mislabeling, e.g., wrong or missing text or figures
- Missing or incorrect information (leaflets or inserts)
- Microbial contamination of non-injectable, non-ophthalmic sterile product with medical consequences
- Chemical/physical contamination (significant impurities, cross-contamination, particulates)
- Mix-up of products in containers (rogues)
- Non-compliance with specification (e.g., assay, stability, fill/weight)
- Insecure closure with serious medical consequences (e.g., cytotoxins, child resistant containers, potent products)

Class III

Class III defects may not pose a significant hazard to health, but withdrawal may have been initiated for other reasons. If deemed relevant by the issuing authority, the Rapid Alert System may be used.

Examples:

- Faulty packaging, e.g., wrong or missing batch number or expiry date
- Faulty closure
- Contamination, e.g., microbial spoilage, dirt or detritus, particulate matter

14.2 Addendum II: Recall Form Example

|                                         |                                                                                                                                           |                      |
|-----------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------|----------------------|
| Section 1                               | Recall Initial Data                                                                                                                       |                      |
| Material Name/Description               | Isperia 5 mg capsules, 30/box, Spain                                                                                                      |                      |
| Lot number (s)                          | 11226-H                                                                                                                                   |                      |
| Associated Investigation or Complaint # | C – 222 & DI – 3346                                                                                                                       |                      |
| Short Description of Reported Event     | The blister film shows leakage in some cases, as if not properly sealed. This causes the capsules to get soft and the contents leaks out. |                      |
| Recorded                                | By: ASBU                                                                                                                                  | Date: 14 NOV 2024    |
| Section 2                               | Product Recall Committee                                                                                                                  |                      |
| Proceed with Product Recall             | Yes / No                                                                                                                                  |                      |
| Summary of Meeting attached             | See Recall Meeting Minutes 14 NOV 2024                                                                                                    |                      |
| Recall Classification                   | Class I / II/III                                                                                                                          |                      |
| Recall Strategy/Actions attached        | See Recall Strategy Isperia 5 mg Spain, 14 NOV 2024                                                                                       |                      |
| Quality Assurance/Qualified Person:     | Name and Signature:<br>J. Menno JMenno                                                                                                    | Date:<br>14 NOV 2024 |



### 14.3 Addendum III: Recall Reconciliation Form Example

|                                                                                                                    |                                               |                                         |                                |
|--------------------------------------------------------------------------------------------------------------------|-----------------------------------------------|-----------------------------------------|--------------------------------|
| <b>Recall Reconciliation Form – Interim Progress Report / Final Reconciliation</b>                                 |                                               |                                         |                                |
| Material Name/Description                                                                                          | <i>Isperia 5 mg capsules, 30/box, Spain</i>   |                                         |                                |
| Lot number                                                                                                         | <i>11226-H</i>                                | Associated Investigation or Complaint # | <i>C – 222 &amp; DI – 3346</i> |
| <b>Units released For Distribution</b>                                                                             |                                               |                                         |                                |
| Units Manufactured (A):                                                                                            | <i>26,000</i>                                 | Units Retained (B):                     | <i>30</i>                      |
| Units for Stability (C):                                                                                           | <i>300</i>                                    | Units Released for Distribution (D)     | <i>25,670</i>                  |
| (E = A – B – C – D):                                                                                               | <i>0</i>                                      | ((B + C + D) / A * 100):                | <i>100%</i>                    |
| <b>Describe any Discrepancy if E ≠ 0 or &lt; 100 %, documentation attached: <i>Yes / No</i></b>                    |                                               |                                         |                                |
| <b>Units Shipped</b>                                                                                               |                                               |                                         |                                |
| Number of Units Shipped (F)                                                                                        | <i>2,000</i>                                  | Distribution Data Attached              | <i>ASBU, 15 NOV 2024</i>       |
| Number of units used (G)                                                                                           | <i>1</i>                                      |                                         |                                |
| Number of units subject to recall (H = F – G)                                                                      | <i>1,999</i>                                  | Number of Units Recalled to Date (I)    | <i>1,999</i>                   |
| <b>Reconciliation (J = H – I)</b>                                                                                  | <i>0</i>                                      | <b>Or ((I – H) / I) * 100</b>           | <i>100%</i>                    |
| <b>Any discrepancies if J ≠ 0 or &lt; 100% need further investigation, documentation attached: <i>Yes / No</i></b> |                                               |                                         |                                |
| Recorded by:                                                                                                       | Name and Signature:<br><i>ASBU</i>            |                                         | Date: <i>23 NOV 2024</i>       |
| Quality Review by:                                                                                                 | Name and Signature:<br><i>J. Menno JMenno</i> |                                         | Date: <i>23 NOV 2024</i>       |



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